

The Evidence-First Research Collective

A charter for working together: shared work, clear ownership and public recognition

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Research first. Evidence always. Build together.

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1. Purpose of this charter

Over the past year, I have been developing a series of research systems as an independent researcher: CARETRACE, ORPHARA, DMOS, RE-FORM X, LUMERA, Q-BenchMed, UMNA, Graphspace, and further work that is still in my archive. Although these may appear to be separate projects, each one is in fact a component of a larger, long-term research programme. Each has been designed to examine one specific idea carefully before it becomes part of that wider effort.

The programme has now reached a stage where it would benefit greatly from a dedicated team. Clinicians, laboratory scientists, statisticians, imaging specialists and engineers would each bring knowledge that can strengthen the work considerably. Thoughtful reviewers and constructive critics are equally valuable, because careful questioning is what turns a promising idea into reliable evidence.

This charter describes the kind of team I hope to build. It rests on three commitments to everyone who joins:

1. **Meaningful work:** real research problems, real responsibility, and the autonomy to do the work well.
2. **Clear ownership:** contributors know what they own, what they lead, and how their intellectual property is protected.
3. **Public recognition:** every contribution is credited by name, and the people who do the work are the people the public sees.

This is a living document. Suggestions and feedback are warmly welcomed, and it will be refined as the collective grows.

2. Current position

- **Research lead.** I serve as the architect and algorithm designer for the programme. My role is to define what each system should do, how it should behave when evidence is limited, and how its performance should be evaluated. Implementation is carried out by my team together with AI coding agents, working from these designs, and every change is reviewed and tested before release.
- **Team.** Two colleagues currently contribute to the work, supporting testing and architecture review.

- **Body of work.** Six projects are available as live, in-browser demonstrations, three are presented as research showcases, and a preprint is available on Zenodo (doi:10.5281/zenodo.23097652). The archive further includes work on protein mutation, biological computation, and a haematology report analysis system for clinical decision support, which is currently in its clinical validation and refinement phase.
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3. What the collective offers

Meaningful work

Contributors work on open research questions with real consequences: how a clinical system should behave when evidence is missing, how imaging evidence can be made traceable, how a benchmark can be made honest. Each person takes responsibility for a defined part of the work, and is trusted to carry it through in their own way.

Clear ownership and protected intellectual property

Every contributor knows what they own and what they lead. Contributions are recorded as they happen, inventions are attributed at the time they are made, and the rights attached to each contribution are written down before work begins. Section 7 sets this out in detail.

Public recognition

The people who do the work are the people who are seen. Contributors are named on project pages, in release notes, in publications and in presentations. Section 8 describes how recognition is given.

4. Roles and opportunities

Rather than looking for specific job titles, the collective welcomes people who would like to take ownership of a part of the work and see it through. The following roles would be especially valuable at this stage.

Research lead and architect. Defines research questions, designs the systems, maintains the coherence of the overall architecture, and takes responsibility for what the group communicates publicly. I currently hold this role.

Subsystem leads. Take responsibility for a specific component, such as an extraction module, an evidence store, a benchmark framework or a reasoning engine. Section 6 describes this

role in more detail.

Research question leads. Take responsibility for a research question, from study design to write-up.

Developers. Design, build, improve and test software. The use of AI-assisted development tools is welcome, provided the contributor understands and can explain the work they submit.

Domain experts. Clinicians, radiologists, haematologists, pharmacists, formulation scientists and biologists, who help ensure the questions are relevant, the assumptions are sound, and the outputs are appropriate and safe.

Testers and validators. Examine the systems rigorously, develop thorough test cases, and compare results against trusted references.

Reviewers and advisers. Review methods and manuscripts and offer candid, constructive feedback. This role is highly valued and can be fulfilled with a modest time commitment.

Data stewards. Oversee data provenance, licensing and appropriate use, and help ensure that sensitive information is handled with care.

Writers and communicators. Help shape results into papers, reports, documentation and public material.

Shared expectations

Across every role, the collective works by a few shared principles:

- Agree on the scope of each contribution, and share any change in availability early.
- Keep a clear record of work done, so that it can be recognised and verified.
- Report results faithfully, including those that do not support the hypothesis.
- Handle data responsibly, and use real patient data only with the appropriate approvals.
- Respect one another's time, expertise and contributions.

5. How we work together

A gentle start. New contributors usually begin with a well-defined task that can be completed in days rather than months, such as a test suite, a methods review or a dataset licence check. This lets both sides see how the collaboration fits.

Flexible commitment. There are no fixed hours. Each contributor indicates the time they can offer, and work is planned around it. If circumstances change, responsibilities can be handed

over smoothly at any time.

Trust and autonomy. Those who lead a piece of work decide how best to carry it out. The emphasis is on outcomes and evidence, and contributors are trusted to manage their own time and approach.

Transparency about the current stage. At present, participation is voluntary. The collective's focus is on producing work of lasting value, and contributors are kept fully informed as new opportunities arise.

Opportunities for students. Students are very welcome. With their institution's agreement, this work may contribute to a project, thesis or portfolio, and support with any required documentation will gladly be provided.

6. Ownership of work

Two complementary forms of ownership help each contributor take real responsibility for their work.

Subsystem leadership

A subsystem lead is responsible for a specific component. Within that component, the lead:

- guides its design and implementation, in line with the shared architecture and the principles in this charter;
- reviews proposed changes and helps protect its stability and quality;
- maintains its documentation, tests and record of known limitations;
- is credited as its named lead in project records, release notes and publications.

Leadership may be shared or transferred. When responsibility changes hands, both contributors and the relevant dates are recorded, and earlier leads keep their credit permanently.

Research question leadership

Some contributors may prefer to lead a research question rather than a component, for example: "Do evidence-guided signals identify errors that confidence scores alone may miss?" The question lead designs the study, sets the success criteria before analysis begins, and leads the write-up. Where the work leads to a publication, the question lead would normally be well placed for first authorship.

7. Intellectual property

Clear intellectual property arrangements protect everyone: the contributor, the collective and the work itself. The principles below guide every contribution. Formal terms are put in writing, with appropriate legal advice, before they are relied upon.

- **Pre-existing work.** Systems, designs and knowledge developed before the collective was formed remain my pre-existing intellectual property. The same applies to every contributor: anything you bring with you remains yours. Pre-existing work is listed openly at the start of a collaboration, so that there is no uncertainty later.
- **Contributions to open-source projects.** These are licensed under the relevant project licence. Contributors retain their copyright, and the licence defines the rights shared with others.
- **Contributions to non-public projects.** Before work begins, contributors sign a short contributor agreement. It sets out the rights granted to the project, the rights the contributor keeps, and how the contribution will be attributed.
- **Inventions and new ideas.** When a contribution leads to something with patent or commercial potential, the people involved are recorded at the time, together with dates and supporting evidence. Inventors are named as inventors, and joint work is recognised as joint work.
- **Attribution that travels with the work.** Wherever a contribution is reused, extended or built upon, its original contributors remain credited.
- **Data rights.** Data remains the property of the organisation or person who provided it, under the terms they set.
- **Future value.** Should the work create commercial value in the future, through licensing, partnerships or a venture, the contribution records kept throughout will form the foundation for recognising contributors fairly, with independent legal advice available to everyone involved.

8. Public recognition

Named credit

Every contributor is acknowledged by name:

- in the project's contributors record, together with their role;
- in the release notes for the work they contributed;

- on the project page and the collective's public site, for substantial contributions, with a short profile if the contributor wishes.

Every contribution is valued, and even a single improvement is recorded.

Authorship by contribution

Authorship reflects each person's actual contribution to the work.

- Contributions are described using the CRediT taxonomy (conceptualisation, methodology, software, validation, formal analysis, investigation, data curation, writing, visualisation, supervision and others), and each publication includes a contribution statement.
- Authorship is extended to those who have made a substantial contribution, taken part in drafting or critically revising the manuscript, approved the final version, and agreed to be accountable for the work. Clinical publications follow the ICMJE recommendations.
- Author order is discussed and agreed in writing before drafting begins, and revisited if contributions change.
- Valued contributions that fall outside the authorship criteria are recognised by name and role in the acknowledgements.
- Authorship is based entirely on contribution, so that recognition is both fair and complete.

Publications and presentations

- **Early dissemination.** Work is shared as preprints (on Zenodo, arXiv, medRxiv or bioRxiv, as appropriate), so that it is public, citable and clearly dated.
- **Complete reporting.** Results that do not support a hypothesis are reported alongside those that do. This openness is already part of the existing benchmark record.
- **Presenting the work.** Presentations, talks and demonstrations are normally given by the question lead or principal contributor, and opportunities are rotated so that team members gain visibility and experience.
- **Clinical review.** Any work with implications for patient care is reviewed by a clinical adviser before it is shared, and its scope and limitations are stated clearly.

References and recommendations

Contributors who would like a reference or recommendation can expect one that is specific and well-founded. Each letter is based on the work itself, including code, reviews, tests and reports, which gives it credibility with universities, employers and collaborators.

9. Open-source approach

The collective is committed to openness wherever it is responsible to do so, because open work is also visible work.

- **Open contributions.** Engines, tools, benchmark frameworks and results are shared under recognised licences (AGPL-3.0, Apache-2.0 or CC BY 4.0, depending on the project). Contributions to open projects are made under the project's licence.
- **Staged release of clinical knowledge.** Clinical knowledge bases and models are released once they have completed validation, so that they are used with confidence and in the right context. This is the stated plan for ORPHARA, among others.
- **Contribution sign-off.** Contributors confirm their right to submit their work through the Developer Certificate of Origin, a simple sign-off on each commit.
- **Respect for third-party work.** The licences of all tools, models and datasets used are respected, and their sources are acknowledged clearly.

10. Governance and decision-making

Governance is designed to be light, transparent and fair.

- **Decisions close to the work.** Subsystem and question leads make day-to-day decisions within their own areas.
- **Shared decisions in writing.** Matters that span several areas, such as architecture, licensing, public communication and authorship order, begin as a short written proposal. Those affected are given time to comment, normally around a week, and the outcome and reasoning are recorded in a decision log.
- **Resolving open questions.** At present, the research lead makes the final decision where consensus has not been reached. As the team grows, this responsibility is intended to pass to a steering group of active leads.
- **Clinical safety review.** A clinical reviewer may pause any public claim or release that raises a patient safety concern, and that review takes precedence.
- **Constructive discussion.** Differences of opinion are a healthy part of research. Discussions focus on the evidence, and where a question remains open, a well-designed experiment is often the best way forward.
- **Code of conduct.** All contributors follow a written code of conduct that ensures a respectful, inclusive and professional environment.

11. How the collective grows

The collective is built around the work first. As the work matures, it is expected to open new possibilities, and each step builds on the recognition and ownership already in place.

Stage	Description	What it means for contributors
1. Collective	A focused team producing evidence and open work	Ownership, named credit, authorship
2. Partnerships	Collaboration with universities, medical colleges, hospitals and industry	Wider visibility, institutional affiliations, access to data and expertise
3. Supported research	Support through research funding, institutional programmes or industry collaboration, where it suits the work	Formal roles, offered first to established contributors
4. Potential venture	Considered where validated work would be best served by a company	Early contributors considered first, guided by contribution records

Each stage is an outcome in its own right. A strong publication, a validated method, an open tool adopted by others, or a lasting partnership are all meaningful achievements for the team.

When external support or partnership is pursued, the people who carried out the work are named in the roles they played, and everyone named reviews the proposal before it is submitted. Many funders and partners work through a host institution, which is one more reason university and medical college collaborators are so valuable to the collective.

12. Our commitments

Contributors can rely on the collective to:

- offer meaningful work with real responsibility and autonomy;
- record ownership clearly, and protect each contributor's intellectual property;
- recognise every contribution publicly, fully and fairly;
- be transparent about the current stage, roles and opportunities;

- represent research findings accurately, and present prototypes as research work until validation is complete;
 - report all results, including those that challenge a hypothesis;
 - treat patient data with the highest standards of care and approval.
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13. Getting involved

If you are interested in contributing, you are warmly invited to get in touch. It would help to include:

- a brief introduction and the area you would like to work on;
- the role from section 4 that suits you best, or another role you would like to propose;
- an indication of the time you are able to contribute;
- a link to previous work, if available, such as code, a publication or a review.

University research groups, medical colleges, independent reviewers and critics are all most welcome. A different perspective is always valued, and thoughtful disagreement is one of the most useful contributions anyone can make.

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This charter will be updated as the collective develops. A version history is maintained alongside the document.